Version:1

Bwrdd Iechyd Prifysgol
Betsi Cadwaladr
University Health Board

ANAESTHESIA, CRITICAL CARE & PAIN MANAGEMENT DIRECTORATE

OBSTETRIC ANAESTHESIA WRITTEN CONTROL DOCUMENT

ANTACID PROPHYLAXIS FOR LABOUR AND DELIVERY

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1. Introduction and Purpose

Gastric emptying is reduced during pregnancy and especially during labour. In pregnancy, lower oesophageal sphincter pressure is reduced and intragastric pressure is increased resulting in a reduction in barrier pressure. The purpose of antacid prophylaxis in labour and prior to Caesarean Section is to reduce the risk of aspiration pneumonitis (Mendelson's Syndrome) among women undergoing Caesarean Section under general anaesthesia. Aspiration pneumonitis carries a high risk of mortality. It's prevention involves the use of the proton pump inhibitor Omeprazole, which inhibits gastric acid production. It also involves the use of sodium citrate which acts as a buffer to neutralise any acid in the stomach.

2. Scope

This guideline is intended as reference for trainee anaesthetists, obstetricians and midwifes seeking a recommended method of antacid prophylaxis for labour and delivery.

3. Points to consider

3.1 Labouring women

Omeprazole 20mg daily orally should be given throughout labour. If the patient needs an emergency Caesarean Section, then **30mls of 0.3M Sodium Citrate orally** should be given as the patient is about to leave the labour room for theatre.

3.2 Elective Caesarean Sections

Women booked for an elective Caesarean Section should receive two doses of **Omeprazole 40mg orally** as a take home prescription following preoperative assessment. One should be taken at **22:00 on the evening prior to surgery**, and the second dose at **07:00 on the day of surgery**. Then prior to surgery, **30 mls of 0.3M sodium citrate** should be given orally as the patient is leaving the ward for theatre.

4. References

1. Gillett GB, Watson JD, Langford RM. Ranitidine and single-dose antacid therapy as prophylaxis against acid aspiration syndrome in obstetric practice. *Anaesthesia* 1984; 39: 638-644